

Emergency First Aid: Fracture Management & Immobilization

Step-by-step instructions for identifying fractures and applying provisional immobilization techniques for upper and lower limbs.

IDENTIFICATION & SYMPTOMS



DIAGNOSTIC IMAGING: In many cases, a fracture is not obvious and can only be confirmed through a professional radiographic (X-ray) assessment.

CRITICAL FIRST ACTIONS



DO NOT MOVE THE VICTIM

Movement can transform a closed fracture into a complicated open one.



CALL FOR PROFESSIONAL HELP

Alert SAMU/Firefighters immediately. Prohibit the victim from moving.



MANAGING OPEN FRACTURES

COVER AND PROTECT:

Use clean cloth or sterile dressing. **NEVER** clean the wound or push bone back.



NO FOOD OR DRINK

Do not give oral intake. Risk of emergency encephalopathy.

SPLINTING & IMMOBILIZATION TECHNIQUES

IMMOBILIZATION MATERIALS

PROFESSIONAL SPLINTS



Pneumatic air splint



Aluminum splint

IMPROVISED / FORTUNE MATERIALS



SECURING THE SPLINT

Tie to the splint, not the limb. Maintain circulation.



THE PADDING RULE

CUSHION THE SPLINT:

Always pad splints with cotton or clothing before application to protect skin and improve stability.



TRACTION:

Alignment Before Splinting if necessary.

UPPER & LOWER LIMB PROTOCOLS

UPPER LIMB (ARM & FOREARM)



Secured with inner and outer splints; use a triangular sling for forearms to secure against the chest.



Elbow at right angle.

LOWER LIMB (LEG & KNEE)



Foot maintained at a right angle.

LOWER LIMB (CROTCH/HIP)



Long outer splint, from armpit and groin to foot

THE "BUDDY" TECHNIQUE



USING THE SOUND LIMB
(When one splint is available).